

Is There A Fragrant Solution To Low Sexual Desire?

Dear Dr. Scantling,

I'm a 52-year-old woman, and I've been suffering with low sexual desire for some time. I just heard that there's a scented patch available without prescription that can increase libido just



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by sniffing it! Do you know anything about this product? Do you recommend it to your patients? Why do you think that we suddenly are hearing so much about sex and the sense of smell?

Ellen, Avon

Dear Ellen,

The subject of smell has been getting some renewed attention with last week's release of Rachel Herz's new book, "The Scent of Desire" (William Morrow). It explores the importance of smell to our survival, emotions, memory and sexual well-being. In the 1990s, Diane Ackerman wrote about the hormonal effects of the smell of musk in "Natural History of the Senses." More than two decades ago, Dr. Gary E. Schwartz's ground-breaking research at Yale found that certain scents, like the smell of spiced apple (grandma's apple pie?), reduced blood pressure and fostered relaxation. So the investigation of smell isn't new.

As far as the product you're asking about, I believe you're referring to Scentuelle. It's a clear patch with a

reportedly "floral" fragrance worn on the inside of the wrist. It's British developers claim that by sniffing the patch each hour, users can increase their libido by mimicking dopamine, one of the feel-good chemicals in the brain. It's well established that elevations in dopamine levels can lead to an improvement in mood, alertness and desire.

What isn't specified, however, is how this product stimulates the dopamine system. I'm not aware of any large-scale randomized controlled studies comparing women sniffing the Scentuelle patch to those smelling a "placebo" patch (although there may be some in the pipeline). I'm hesitant to make any clinical recommendations until there is some well-substantiated data about how it works.

But because I was curious, I decided to do my own "research" — on me. I ordered a box of 30 for about \$35 from www.myscentuelle.com. The package arrived in less than a week. I immediately placed one of the small spherical patches my wrist, as directed, and took a sniff. It didn't smell floral to me. It reminded me of baby powder. "Not an especially sexy association," I thought.

My first challenge was to identify what response I was actually looking for. What is increased desire supposed to feel like? Should I feel a sudden urge to call my husband for an afternoon "delight?" Was I supposed to feel heat . . . stirrings . . . fantasies . . . what? I can't honestly say I noticed anything unusual

I continued to dutifully sniff my wrist and reflect on my libido each hour. After eight hours and eight sniffs, I quit. So I guess you could say that I didn't give it a fair try (reports claim the product begins to "work" from the very first sniff, with a cumulative effect over a few weeks). I was also at my office, which undoubtedly reduced my potential ardor.

Then a thought hit me. Perhaps the specific scent is less important than just thinking about sexual desire every hour. Hmmm. Maybe a woman's libido would increase if she set a timer to ring on a regular basis and then thought of something or someone sexy? It's certainly worth a try — and it wouldn't cost you \$35 a month! If you do this fun experiment, please let me know!

We know that sexual interest is multi-determined. Hormones, blood flow, nerve conduction, medications, anxiety, depression and tons of other things can affect desire. And yes, what we think about and how often we think about it has a huge impact on our sexual receptivity.

Before you can improve your libido, Ellen, you need to understand what's contributing to your "suffering." You don't say how long you've had this problem and whether things are getting worse. In what situations, if any, do you feel more interested in sexual intimacy? Do you also have difficulty with arousal (lubrication)? It's not unusual for women who have decreased arousal to lose interest in sex because it's often uncomfortable or uninspiring. Desire,

arousal and orgasm are distinct responses, but they affect one another positively and negatively.

I'm doubtful that we will ever discover a sure-fire "scent" to repair our sexual problems. Satisfying sexuality is more complex, and that's something to be thankful for. By attempting to reduce our passion to a set of pheromones, I believe we do ourselves a disservice.

Dear Dr. Scantling,

I'm 46, and my doctor is recommending a hysterectomy. I've always had great orgasms and wonderful sex, and I don't want this to change. Will a hysterectomy hurt my sex life?

Cheryl, Bristol

Dear Cheryl,

This is a good question, and the short answer is: "It depends." There are women who complain that they feel something is missing after their hysterectomy and describe a decrease in orgasmic fullness. Other women with large fibroids or bleeding report more sexual pleasure after the removal of the source of pain and discomfort.

There's no question that the uterus and the cervix play a role in the physiology of orgasm, but its importance varies from woman to woman.

If there is surgical damage to the pelvic nerves, it can interfere with the nerve supply to the blood vessels responsible for the swelling and lubrication of arousal. Whether the cervix is removed or vagina is

"shortened" can also affect sexual pleasure.

Many women are afraid they will lose their femininity after hysterectomy. If the ovaries and fallopian tubes are removed along with the uterus, there will be a decrease in circulating estrogens and androgen, causing a surgical menopause. Without hormone replacement, you will experience all of the hot flashes and symptoms of a naturally occurring menopause — just more suddenly.

The fact that you have "great orgasms" and "wonderful sex" indicates that you are (and are likely to continue to be) a sexually responsive woman. Anxiety about altered sexual functioning can be a self-fulfilling prophecy. Information is power. Continue to do your homework, and make sure to discuss all of your concerns and options with your doctor or nurse practitioner.

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